

Ethnicity influences breast cancer risk, diagnosis, and treatment but not survival, study finds

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ABSTRACT

The authors wrote: "Although there are differences in recorded stage of disease at diagnosis and treatment between ethnic groups, it is encouraging that there is little variation in survival after adjustment for these factors, suggesting that some of the survival differences found in previous US studies may reflect the problems some groups have in accessing effective treatments in US healthcare systems."

FULL TEXT

A woman's risk of getting breast cancer, the stage at which it is diagnosed, and the treatment she gets all vary with ethnic background, a new UK study has found.

But the researchers found, after adjusting fully for age, level of socioeconomic deprivation, stage of disease, and treatment received, that survival from breast cancer did not vary significantly (*British Journal of Cancer* doi: 10.1038/sj.bjc.6604852).

The researchers, from King's College London, analysed data on 35 631 women who received a diagnosis of breast cancer in southeast England between 1998 and 2003. The ethnic groups they looked at were white, Indian, Pakistani, Bangladeshi, black Caribbean, black African, and Chinese.

They found that white women had the highest age standardised incidence of breast cancer and Bangladeshi women had the lowest. Incidence rate ratios calculated with that among white women as the baseline were all significantly lower: Indian 0.68 (95% confidence interval 0.64 to 0.73), Pakistani 0.59 (0.51 to 0.69), Bangladeshi 0.23 (0.20 to 0.26), black Caribbean 0.80 (0.74 to 0.86), black African 0.66 (0.59 to 0.74), and Chinese 0.54 (0.47 to 0.63).

Their analysis also showed that the differences in incidence between white women and the other ethnic groups increased with age group, a pattern most evident in Bangladeshi, black Caribbean, black African, and Chinese women.

White women were the most likely to have a stage recorded at diagnosis (75% of this group) and Bangladeshi women the least likely (55%). After patients whose stage was unknown were excluded, the researchers found that Pakistani women were the most likely to be given a diagnosis of metastatic disease (17%). White women were less likely than all the other groups to have a diagnosis of metastatic disease (7%), although the numbers in two groups, Bangladeshi and Chinese women, were too small for the difference to be significant.

"These findings suggest that a strategy of earlier detection should be pursued in black and South Asian women," wrote the authors.

Black African women were less likely than white women to have a record of surgery for cancer (63% versus 72%; $P<0.003$) and hormone therapy (32% v 54%; $P<0.001$) and more likely to receive chemotherapy (38% v 29%; $P=0.001$). Pakistani women were less likely than white women to receive radiotherapy (27% v 36%; $P=0.043$) and hormone therapy (41% v 54%; $P=0.014$), whereas black Caribbean women were less likely than white women to receive hormone therapy (39% v 54%; $P<0.001$).

The authors wrote: "Although there are differences in recorded stage of disease at diagnosis and treatment between ethnic groups, it is encouraging that there is little variation in survival after adjustment for these factors, suggesting that some of the survival differences found in previous US studies may reflect the problems some groups have in accessing effective treatments in US healthcare systems."

They added, "With better access to earlier detection, and the possibility of more aggressive treatment, survival could be improved for all women."

DETAILS

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